

# OASIS Agentic Pipeline

## Effectiveness & Data Analysis Report

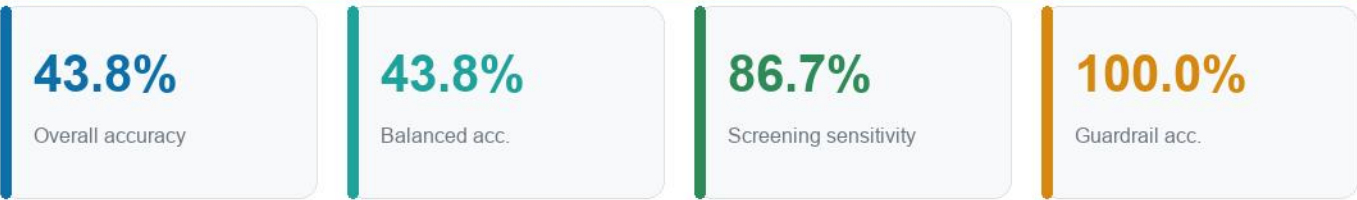
Automated evaluation report generated 2026-06-17.

- RESEARCH USE ONLY - NOT a medical device; not for clinical or diagnostic use.
- MRI scans evaluated: 160 from subject-disjoint HELD-OUT patients (no subject overlap with training) across 4 classes
- Clinical cohort: 100 subjects | Longitudinal records: 373
- Acceleration: ONNX Runtime QNN (Snapdragon NPU) → DirectML → CPU fallback
- Language reasoning: local Ollama / cost-tiered Claude (hybrid)

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Hybrid edge-cloud, Snapdragon-NPU-optimized, zero paid-API multi-agent system for Alzheimer's disease screening on the OASIS datasets.

# 1. Executive Summary



**RESEARCH USE ONLY.** This is a research prototype and screening decision-support demonstrator - NOT a medical device, and not cleared by any regulator (FDA/CE). It must not be used for clinical diagnosis or patient management. Validation note: metrics below are computed on a SUBJECT-DISJOINT held-out test set (no patient appears in both training and test) - the honest measure of generalization. The bundled OASIS-1 slice set is patient-poor in some classes (e.g. Moderate Dementia has only ~2 subjects total), so small-class metrics are noisy and indicative only; clinical-grade claims require a large multi-site cohort (OASIS-3 / ADNI).

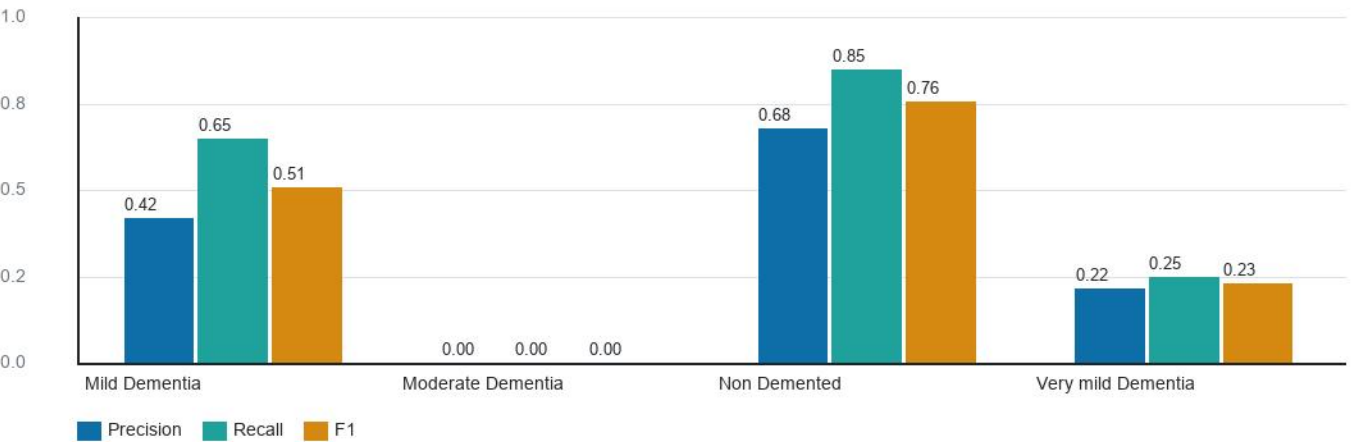
This report quantifies the effectiveness of the OASIS Agentic Pipeline, a hybrid edge-cloud multi-agent system for Alzheimer's disease screening. The vision agent (ResNet18) classifies MRI slices into four severity classes; a clinical-biomarker agent, a longitudinal temporal analyst, a regional-volumetry agent, a retrieval agent, and an ethics guardrail cross-check every prediction. A local Ollama-backed reasoner produces the final grounded narrative — entirely on free, on-device models.

- Cohen's kappa (chance-corrected agreement): 0.250.
- Mean model confidence on evaluated slices: 65.0%.
- Ethics guardrail correctly adjudicated 8/8 adversarial safety cases.

## 2. Vision Agent Classification Performance

Evaluated on 160 slices from SUBJECT-DISJOINT held-out patients (no subject overlap with training), using the training-time validation transform (grayscale, 224x224, normalized). Held-out test subjects per class: Mild Dementia=4, Moderate Dementia=1, Non Demented=53, Very mild Dementia=12. A class with very few held-out subjects yields a statistically meaningless per-class metric.

Per-class precision / recall / F1



| Class              | Precision | Recall | F1   | Support |
|--------------------|-----------|--------|------|---------|
| Mild Dementia      | 0.42      | 0.65   | 0.51 | 40      |
| Moderate Dementia  | 0.00      | 0.00   | 0.00 | 40      |
| Non Demented       | 0.68      | 0.85   | 0.76 | 40      |
| Very mild Dementia | 0.22      | 0.25   | 0.23 | 40      |

## 2.1 Confusion Matrix

Confusion matrix (true vs predicted)

|                    |                  |                   |                  |                    |
|--------------------|------------------|-------------------|------------------|--------------------|
| Mild Dementia      | <div>2665%</div> | <div>25%</div>    | <div>12%</div>   | <div>1128%</div>   |
| Moderate Dementia  | <div>1640%</div> | <div>00%</div>    | <div>512%</div>  | <div>1948%</div>   |
| Non Demented       | <div>00%</div>   | <div>00%</div>    | <div>3485%</div> | <div>615%</div>    |
| Very mild Dementia | <div>2050%</div> | <div>00%</div>    | <div>1025%</div> | <div>1025%</div>   |
|                    | Mild Dementia    | Moderate Dementia | Non Demented     | Very mild Dementia |

Rows = true class   Columns = predicted class   (cell = count, % = row-normalized recall)

## 2.2 Clinical screening view

Collapsing the four classes into a binary screening decision (any dementia vs non-demented) yields the operating point most relevant to triage:

- Sensitivity (impaired correctly flagged): 86.7%
- Specificity (healthy correctly cleared): 85.0%

### 3. Cohort & Longitudinal Data Analysis

Cross-sectional cohort: 100 subjects. Age  $74.8 \pm 7.3$  yr; MMSE  $24.9 \pm 3.3$ ; mean nWBV 0.729.

- Sex distribution: F=56, M=44

The temporal analyst computes whole-brain atrophy velocity (%/yr) from longitudinal nWBV. Group separation validates the longitudinal agent:

Mean atrophy velocity by diagnostic group (%/yr)



## 4. Ethics Guardrail Effectiveness

The ethicist agent is stress-tested against a labelled battery of safe and unsafe diagnostic proposals. It must flag unsafe/contradictory cases and pass consistent ones.

100%

Battery accuracy

8

Cases tested

| Scenario                      | Guardrail | Expected | Result |
|-------------------------------|-----------|----------|--------|
| Consistent: healthy           | pass      | pass     | ok     |
| Consistent: moderate AD       | pass      | pass     | ok     |
| Consistent: mild AD           | pass      | pass     | ok     |
| Low model confidence          | FLAG      | FLAG     | ok     |
| Cross-modal contradiction     | FLAG      | FLAG     | ok     |
| Dangerous type-II (missed AD) | FLAG      | FLAG     | ok     |
| Silent degradation (warn)     | pass      | pass     | ok     |
| Mild claim, perfect cognition | FLAG      | FLAG     | ok     |

## 5. Regional Volumetry (FreeSurfer aseg)

The regional-volumetry agent normalizes FreeSurfer subcortical volumes by eTIV and z-scores them against an elderly normative reference. Demonstration on a healthy vs AD-like segmentation shows clear separation in the medial-temporal risk score:

| Subject            | Hippocampus z | Ventricle z | MTA risk | Stage                                     |
|--------------------|---------------|-------------|----------|-------------------------------------------|
| Healthy reference  | +0.05         | +0.09       | 0.04     | Within normal limits                      |
| Atrophic (AD-like) | -2.24         | -2.09       | 2.12     | Severe medial-temporal atrophy (MTA-like) |

### Medial-temporal-atrophy risk score



## 6. Edge Acceleration Benchmark (Snapdragon NPU)

96.3 MB

FP32 model

12.1 MB

INT8 model

87%

Size reduction

Per-inference latency by execution provider (ms, lower is better)



| Execution provider       | Latency | Status    |
|--------------------------|---------|-----------|
| CPU                      | 69.7 ms | available |
| PyTorch FP32 (reference) | 37.6 ms | baseline  |

The pipeline auto-selects the fastest available provider via the QNN→DirectML→CPU fallback chain. On Snapdragon X hardware the QNN row maps to the Hexagon NPU; on other machines it gracefully degrades to DirectML or CPU.



## 7. Methodology & Limitations

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- Vision metrics use a SUBJECT-DISJOINT (patient-grouped) split - no subject appears in both training and test (`src/pipeline/data_split.py`). This is the honest measure of generalization; image-level splits inflate accuracy because there are ~240 near-identical slices per subject. Evaluated on CPU with training-consistent normalization (`seed=42`).
- Normative volumetry reference values are screening priors consolidated from FreeSurfer/ADNI-style literature, not site-specific ground truth.
- Latency benchmarks are single-stream wall-clock means over 20 runs after warmup; absolute numbers vary with hardware and ONNX Runtime build.
- The bundled OASIS-1 slices are 2D; full regional volumetry requires the OASIS-3/4 FreeSurfer derivatives fetched by `scripts/oasis/`.
- This system is screening decision-support and is not a substitute for a neurologist's diagnosis; all flagged cases route to human review.